

إسم المريض ربيعاً : سماح وجيه ليب جيد

الحى / المدينة : العمران

المحافضة : الحيز ه

رقم البطاقة : 28505122502007 رقم التلفون : 01221120646

العمر(تاريخ الميلاد) : 12-05-1985 المهنة : ربه منزل

أقر انا الموقع أدناه انني لا اتمتع باي نظام علاج مثل:- التأمين الصحي-القوات المسلحة-الشرطة-
الهيئات-المؤسسات او اي نظام يتعارض مع نظام العلاج علي نفقة الدولة- وسوف اتحمل كافة
نفقات العلاج في حالة ثبوت غير ذلك

الإخصائي الاجتماعي (-----)

المقرر بما فيه التوقيع: (-----)

التشخيص الإكلينيكي : سرطان بالثدي (مبادرة)

نتائج الأبحاث : (مرفق)

التشخيص النهائي : سرطان بالثدي (مبادرة)

خطة العلاج : علاج مبادرة سرطان الثدي تراستوزوماب تحت الجلد [14112] **قسم العلاج :** الاورام

التعليق على التشخيص : -----

الاسم: د/محمد كيلا ني
 الاسم: د/ندي حسن
 الاسم: د/نرمين رمضان

الوظيفة: _____
 الوظيفة: _____
 الوظيفة: _____

التوقيع: _____
 التوقيع: _____
 التوقيع: _____

(مختبر)

تتعمد صحة البيانات المرفقة
رئيس اللجنة ومدير
المستشفى.

أطلعت اللجنة علي لائحة العلاج الخاصة بالمجالس وتلتزم جهة العلاج بلائحة المجالس الطبية. كما تقر اللجنة بأن البيانات المدونة في هذا النموذج صحيحة.

تتعهد المستشفى أن لا يتم تنفيذ القرار إلا بعد مراجعة الملف الطبي للمريض وإعادة مناظرة المريض بالمستشفى قبل الصرف.

----- / د ----- / د ----- / د. | ----- / د. |

قرار اللجنة

التشخيص النهائي :

خطة العلاج :

المستشفى :

جهة العلاج : --

ملاحظات : ---

ختم

شعار الجمهورية

مدير عام

مديرية الشؤون الصحية

رئیس

المجلس الطبي المتخصص

--

Name: سماح وجيه ليب جدد National ID: 28505122502007	
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تقرير طبي

السيد
تحية طبية وبعد,,

إيماء للطلب المقدم من	بشأن تقرير طبي للمريض
بقسم:	رقم طبي:

نتشرف بان نوضح فيما يلي تقريرنا عن حالته
--

تاريخ وساعة الدخول : -- --:-- yyyy-mm-dd	تاريخ وساعة الخروج : PM 01:17 05-07-2026
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وبعد الكشف والفحوصات والمتابعة وجد انه
--

المریضة تعاني من ورم غير حمید بالثدي و تحتاج الي Trastuzumab Subcutaneous

التاريخ : 05-07-2026	الوقت : -- --:--
	الطبيب المعالج : د/شيماء لاشين



05/25/2025

2

MDT FORM

Name:	Semah Wageh Labib Ged	MRN:	2820
Age:	41 Y	ID:	28505122502007
Menopausal Sta:	prem	Phone:	01276036647
NRS SCORE:		Marital Status:	
BMI:		Family History:	
CO Morbidity:		Reason Of MDT:	

Clinical Data:

BX: R L
 ER: PR: HER2: KI67:

Sonomammography:

Mets Workup:

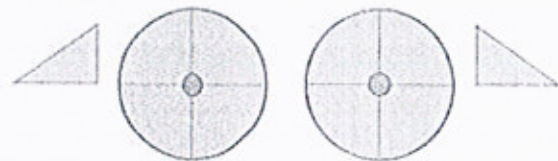
History:

prem rt breast cancer rt ssm and alnd on 7/3/2026 t3 n2 6/13 mo by cts ldc
 2 er5/6 pr4/8 her23+ ef 67% true cut biopsy was dcis so there was no her2 in
 true cut biopsy mdt for 3 ac till result of her 2 on mastectomy as pt is delayed
 to start first ac till remdt for mdt for antiher2 and plan of neoadj tt

Recommendations:

MDT Outcome & Plan

Adj AC 4 LYCLRS Followed by Herceptin- Taxol 12 weeks
 Herceptin Maintenance complete 1y
 PORT
 ET Zoladex + AI 5-7y



T N M

SIGNATURE

MEDICAL ONCOLOGY	SURGERY	RADIOLOGY	PATHOLOGY	Nutrition	Psychiatric
د/ نورهان مصطفى	د/ نوري حسن				

د/ نوري حسن 2025/05/25



Name: سماح وجيه لبيب جيد MR. : 2820	
Detailed Medical Report	
السيد	
تحية طبية وبعد,,	
إيماء للطلب المقدم من	بشأن تقرير طبي للمريض
بقسم:	رقم طبي:
ننشرف بأن نوضح فيما يلي تقريراً عن حالته	
تاريخ وساعة الدخول :	تاريخ وساعة الخروج :
وبعد الكشف والفحوصات والمتابعة وجد انه The patient presents with a history of right breast cancer, staged T3 N2 M0, with a mastectomy performed on 7/3/2026. The initial pathology was ductal carcinoma in situ (DCIS) with no HER2 amplification on true-cut biopsy; HER2 testing on the mastectomy specimen was pending. The multidisciplinary team recommended adjuvant anthracycline-based chemotherapy (AC) for three cycles until HER2 results were available, followed by trastuzumab (Herceptin) and taxane (Taxol) for 12 weeks. The patient was delayed in starting AC until the mastectomy was completed, and a new breast-specific regimen was planned for cycle 1 of AC with adriamycin 105 mg/m² and epirubicin 1050 mg/m². The patient received four cycles of AC as per the MDT plan, followed by trastuzumab and taxane for 12 weeks. Trastuzumab maintenance was completed for one year, and the patient is scheduled to receive PORT, Zoladex, and an aromatase inhibitor for 5–7 years. On 04/06/2026, the patient received cycle 2 of AC after laboratory values were reviewed; the next visit was scheduled for 25/6/2026. A bone scintigraphy performed on 26/5/2026 showed no bone metastases. On 06/08/2026, the patient began cycle 1 of trastuzumab (SC) and week 1 of taxol 135 mg, with week 2 planned for 3-phase adjuvant therapy. The next appointment was to review echocardiography results. On 13/08/2026, the patient attended clinic for a urine culture/sensitivity, which was contaminated. The patient was scheduled for week 2 of taxol and a follow-up clinic next week to assess dysuria. On 20/08/2026, mild dysuria was noted; the patient was on week 3 of taxol. The next clinic visit was planned for urine culture/sensitivity and an echocardiogram. An echocardiogram on 13/8 showed an ejection fraction of 66%. On 27/08/2026, the patient's ALT was 106 U/L (grade 2). The patient received a 10% dose reduction of taxol. The patient was on week 4 of taxol, cycle 2 of trastuzumab, and week 5 and 6 of a 3-phase regimen with liver support. The patient has completed 2 cycles of AC and 6 weeks of taxol to date.	
تحريراً في ,	
التاريخ :	21-09-2026
الطبيب المعالج :	dina makram
يعتمد ,	

Name : Samah Wajih Labib Jied

Date : 26-05-2026 06:42

Branch : Dokki

Medical No. : 382575

Ref.Physician Prof : Goustav Roussey Egypt

Tc^{99m} MDP SKELETAL SCINTIGRAPHY**History**

41 year-old female patient , presented with history of surgically treated right breast cancer, received chemotherapy, complains of bony ache and referred for metastatic work up .

Technique

- Anterior and posterior whole body scans were acquired 10 minutes and 2 hours post injection of 20 mci of the tracer.

Findings

- No evidence of tracer uptake abnormalities that would account for bone secondaries could be seen.
- Degenerative changes are noticed at different levels of the spine and around big joints.

IMPRESSION

- No definite osseous metastases detected on scintigraphic basis.

Much obliged
Dr. Alaa Nagy, MD
Nuclear medicine

SAS

Patient Name : الأستاذ / سماح وجه لبيب جيد	Request No. : 13005331
Patient No. : 28505122502007	Request Date : 07/03/2026
Path No. : 633/26	Verification Date : 12/03/2026
Sex : Female	Financial Category : مبداءه - مبداءه
Age : 40 Y	Department : Out Patient
Referring Doc :	Registered by : marwa mohamed youss

Pathology Report

Clinical Data: Right breast cancer, Up-Front.

Nature of Specimen: Right simple mastectomy, Sentinel lymph nodes biopsy & Completion axillary lymph nodes dissection.

Gross Examination:

- * Right simple mastectomy specimen measured 18x16x5 cm, with covering skin ellipse measured 18x10 cm. Nipple & areola looked unremarkable grossly.
- * Serialling showed a mass 5.5x4x2 cm, located 2.5 cm from the least deep margin.
- * SLNs: Fibrofatty tissue piece measured 11x6x1 cm, entangling eight lymph nodes (Only 5 submitted for frozen), the largest measured 5x2x1 cm.
- * Completion axillary lymph nodes: Axillary fat measured 7x6x2 cm, entangling 5 lymph nodes, the largest is 2x1x0.5 cm.
- * Total-nodal count is; Thirteen lymph nodes.

16 Blocks:

- 1:10- Mass., 11- N&A., 12- Deep margin., 13:14- SLNs., 15:16- ALNs.
- Gross was done by; Dr. Heba El-Shafeey.

** Frozen (intra-operative) results:

- SLNs: Positive metastatic deposits in five out of five sentinel lymph nodes (5/5) (Eight lymph nodes are detected but only 5 submitted for frozen).
- ** Frozen was examined by; Prof. Dr. Gamal Abd-El-Aty.

Microscopic Examination:

- * Histological Type: Invasive duct carcinoma.
- * Grade 2 (differentiation 2, nuclear pleomorphism 2, mitosis 2).
- * Ductal carcinoma in situ: Extensive high grade ductal carcinoma in situ (Solid & cribriform pattern with comedo type necrosis).
- * Angioinvasion: Present.

Verified by :

Dr.Duaa Mohamed

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Patient Name : الأستاذ / سماح وجه لبيب جيد	Request No. : 13005331
Patient No. : 28505122502007	Request Date : 07/03/2026
Path No. : 633/26	Verification Date : 12/03/2026
Sex : Female	Financial Category : جوماتاف روسي - مبادره
Age : 40 Y	Department : Out Patient
Referring Doc :	Registered by : .marwa mohamed youss

Pathology Report

- * Perineural invasion: Absent.
- * Microcalcification: Present.
- * Nipple & areola: Free.
- * Deep margin: Free.
- * Sentinel lymph nodes: Positive metastatic deposits in five out of eight sentinel lymph nodes (5/8) with no extra-nodal extension.
- * Completion axillary lymph nodes: Positive metastatic deposits in one out of five lymph nodes (1/5) with no extra-nodal extension.
- * Total nodal status: Positive metastatic deposits in six out of thirteen lymph nodes (6/13) with no extra-nodal extension..

Diagnosis:

- Right breast cancer (Up-front), Right simple mastectomy, Sentinel lymph nodes biopsy& Completion axillary lymph nodes dissection:
- Invasive duct carcinoma, Grade 2.
 - Extensive high grade ductal carcinoma in situ (Solid & cribriform pattern with comedo type necrosis).
 - Nipple & areola: Free.
 - Deep margin: Free.
 - Total lymph nodal status: Positive metastatic deposits in six out of thirteen lymph nodes (6/13) with no extra-nodal extension.
 - Stage: pT3, pN2a.

Signature

Dr. Walaa Ibrahim Ghoneim.

Verified by :

Dr.Duaa Mohamed

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Patient Name	: الأستاذ / سماح وجه لبيب جيد	Request No.	: 13005331
Patient No.	: 28505122502007	Request Date	: 07/03/2026
Path No.	: 633/26	Verification Date	: 28/04/2026
Sex	: Female	Financial Category	: جوماتاف رومي - مبادره
Age	: 40 Y	Department	: Out Patient
Referreing Doc. :		IHC No.	:
		Registered by	: .marwa mohamed yousse

IMMUNOHISTOCHEMISTRY REPORT

Clinical Data: Right breast cancer (Up-front) diagnosed invasive duct carcinoma grade 2 with extensive DCIS, stage; pT3, pN2a.

Nature of Specimen: Right simple mastectomy, Sentinel lymph nodes biopsy & Completion axillary lymph nodes dissection.

Technique:

- Sections were prepared from the paraffin block, processed in Bench Mark ULTRA machine (Ventana) and stained by rabbit monoclonal antibodies using ultra view HRP DAB as chromogen and hematoxylin as counter stain. The sections treated against:
- ER clone (1E2), PR clone (SP1) and Her2 clone (4B5)

Results:

- * IPT (All controls showed appropriate reactivity):
- ER: Strong nuclear reaction in 10% of tumor cells (score 5/8).
- PR: Moderate nuclear reaction in 2% of tumor cells (score 4/8).
- Her2: Strong complete membrane reaction in >10% of tumor cells (score 3+).

Conclusion:

Right breast cancer (Up-front), Right simple mastectomy, Sentinel lymph nodes biopsy & Completion axillary lymph nodes dissection:

- Invasive duct carcinoma, Grade 2.
- Extensive high grade ductal carcinoma in situ (Solid & cribriform pattern with comedo type necrosis).

IPT:

- ER: Positive (score 5/8).
- PR: Positive (score 4/8).
- Her2: Positive (score 3+).

Verified by :

Dr.Duaa Mohamed

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Patient Name الأستاذ / سماح وجه لبيب جيد
Patient No. 28505122502007
Path No. 633/26
Sex Female
Age 40 Y
Referring Doc.

Request No. 13005331
Request Date 07/03/2026
Verification Date 28/04/2026
Financial Category جوسف رومي - مبادره
Department Out Patient

Registered by .marwa mohamed yousse

IMMUNOHISTOCHEMISTRY REPORT

* Scoring system: "ER, PR"

ASCO-CAP ER/PR Test Guideline Recommendations: proportion of cells staining (0, no staining; 1, >1%; 2, between 1% and 10%; 3, between 11% and 33%; 4, between 34% and 66% and 5, between 67%-100% of the cells staining). Intensity of positive tumor cells (0, none; 1, weak; 2, intermediate; and 3, strong). Total score from 0 to 8.

* Scoring system: "Her2"

Reference: ASCO-CAP Her2 Test Guideline Recommendations. IHC 0 (Null): as defined by no staining observed. IHC 0+ (Ultra-low) as defined by membrane staining that is incomplete and is faint/barely perceptible and within < 10% of the invasive tumor cells. IHC 1+: as defined by incomplete membrane staining that is faint/barely perceptible and within > 10% of the invasive tumor cells. IHC 2+: based on circumferential membrane staining that is incomplete and/or weak/moderate and within > 10% of the invasive tumor cells or complete and circumferential membrane staining that is intense and within < 10% of the invasive tumor cells. IHC 3+: based on circumferential membrane staining that is incomplete, intense > 10% of tumor cells.

Signature

Dr. Walaa Ibrahim Ghoneim.

Verified by :

Dr. Duaa Mohamed

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ECHO REPORT

NAME: ساح وجہ

DATE: 14 / 5 / 2026

ID NUMBER: /

SEX: ♀ AGE: 41

EF	<u>67%</u>	55-75%	PWT	<u>0.8</u>	0.6-1.1cm
LVEDD	<u>4.1</u>	3.5-5.8cm	AO.R	<u>2.5</u>	3-3.7cm
LVEDS		2-4cm	LA	<u>3.3</u>	2-4cm
FS		25-45%	RV		0.7-2.3cm(supine)
SWT	<u>0.9</u>	0.6-1.1cm	AO.CUSP		1.5-2.6cm

LV Dimensions ane

LV Wall Thickness ane

LV Diastolic Function ane

SWMA ane

LA and Ao Root Dimensions ane

Mitral Valve ane

Aortic Valve ane

Tricuspid Valve ane

Pulmonary Valve ane

Right Side and Systolic Function ane

Intracardiac Masses and Effusion ane

DR.

Dr. J. J. J.

جمهورية مصر العربية
بطاقة تحقيق الشخصية

سماح
وجيه لبيب جيد

١ امتداد النوبة - الطالبيه - الهرم
العمرانية - الجيزه

١٩٨٥/٠٥

٢ ٨٥ ٠٥ ١٢ ٢٥ ٠٢٠ ٠٧

LJ2678722

٢٠٢٥/٠٩ ٢٨٥٠٥١٢٢٥٠٢٠٠٧

ربة منزل

أنثى
مسيحية
متزوجة

عادل كامل قلته جورجي

البطاقة سارية حتى ٢٠٣٢/٠٩/١٥

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